Supplement Table 7: Findings of studies examining the association between occupational sitting and other cardiometabolic risk markers [1]

Nr.	Study	Sample	Major occupational groups	Length of follow-up/ duration	Assessment of occupational sitting (as analysed)	Outcomes[2]	Findings[3]	Study quality[4]	Sex-stratified analysis
	First author, year, country, cohort	n (% female) age at baseline	General population or specifica-tion		Categories of measurement and standardized instrument (if applicable)	(Data basis)	Statistical results and reference categories (e.g. risk estimates with 95 % confidence interval)	(++) (+) (0)	Yes/no
1.	Picavet 2016 Netherlands Doetinchem Cohort Study	n=1509 (% F not reported); 20–60 y	Paid employment and self-employed, general population (incl. working)	(1987–2010) 15 y	Self-reported OPA in 4 categories as sedentary work, mainly standing, manual, or high physical loads at 5 time points, categorized into stable sitters (sedentary at least at 3 out of 5 measurements) and non stable sitters at work	Hypertension, hyper-cholesterolemia, blood pressure, total cholesterol (measured/ objective)	Reference group=stable non sitters at work hypertension OR=1.08 (0.84–1.39) increased cholesterol OR=0.80 (0.60–1.07) **Stable sitting at work was not associated with any cardiometabolic health outcome, e.g. not with hypertension or increased cholesterol.**	+	No
2.	Saidj 2015 Denmark Health 2006 cohort	n=1403 (54 % F); 18–69 y mean age: 44 y	General population (incl. working)	(2006–2011) 5 y	Physical Activity Scale 2 (PAS2), assessing self-reported time spent in daily sedentary work (h/d)	Waist circumference (WC), high density lipoproteine (HDL)-cholesterol, triglycerides, insulin level, blood pressure (objective/ measured)	Risk estimate WC β=-0.02 (-0.06–0.02) (p<0.05) **Work sitting predicted decrease of 0.13 cm per additional sitting hour WC; no associations with other metabolic health marker**	+	No
3.	Stamatakis 2017 Great Britain Whitehall II cohort study	n=4811 (27.2 % F); mean age: 43.8 y	Clerical and office support, executive, senior adminitratives grades	(1997–2011) mean 13 y	Self-reported work sitting time/wk summed up into 3 categories: 0–≤15 h/wk; 15–<35 h/wk; ≥35 h/wk	Incident diabetes (measured/ objective)	Reference low work sitting 0 to <15 h/wk HR work sitting 15–<35 h/wk: HR=1.14 (0.87–1.51) work sitting >35 h/wk: HR=1.14 (0.87–1.51) **Work time sitting not associated with incident diabetes.**	+	No

Legend:
n (% F): number of participants (percent female)
OPA: occupational physical activity
1 In alphabetical order and after methodological assessment rating (high to low quality)
2 Waist circumference (WC), high density lipoproteine (HDL)-cholesterol
3 In model adjusted for age, sex, leisure time physical activity (LPTA), health behaviour (alcohol, smoking, diet), and socioeconomic status (education, household income) if available, unless reported otherwise
4 Methodological Assessment incl. risk of bias: assessment after the SIGN-checklists resulting in overall ratings of 'high quality' (++), 'acceptable quality' (+) and 'unacceptable/low quality' (0), synonymic with 'high quality' (++) / low risk of bias, 'acceptable quality' (+) / accepable risk of bias, and 'unacceptable/low quality' (0) / unacceptable risk of bias.